FORM I-140 IMMIGRANT PETITION FOR ALIEN WORKERS

EB-2 NATIONAL INTEREST WAIVER — PETITION COVER LETTER

Manasa Jampani — DRAFT FOR ATTORNEY REVIEW

> **ATTORNEY USE NOTE:** This document is a detailed factual and legal narrative prepared by the petitioner for attorney review and finalization. All legal arguments, citations, and factual assertions must be independently verified by licensed immigration counsel before filing. Sections marked [ATTORNEY TO VERIFY] require particular attention. Statistics cited herein should be verified against the original source documents included in the evidentiary exhibits.

[Date of Filing]

Director
USCIS Nebraska Service Center [ATTORNEY TO VERIFY — confirm correct service center for I-140 EB-2 NIW at time of filing]
850 S Street
Lincoln, NE 68508

RE: Form I-140, Immigrant Petition for Alien Workers
Preference Category: Employment-Based, Second Preference (EB-2)
Basis for Waiver: National Interest Waiver Pursuant to INA § 203(b)(2)(B)(i)
Petitioner: Manasa Jampani
Matter of Dhanasar, 26 I&N Dec. 884 (AAO 2016)

Dear Director:

This brief is respectfully submitted on behalf of **Manasa Jampani** in support of her Form I-140, Immigrant Petition for Alien Workers, seeking classification as an employment-based second preference (EB-2) immigrant with a National Interest Waiver (NIW) of the otherwise applicable job offer and labor market test requirements. The waiver is sought pursuant to Section 203(b)(2)(B)(i) of the Immigration and Nationality Act, 8 U.S.C. § 1153(b)(2)(B)(i), and evaluated under the three-prong analytical framework set forth in **Matter of Dhanasar, 26 I&N Dec. 884 (AAO 2016)**.

For the reasons set forth below, and as supported by the accompanying evidentiary exhibits, all three prongs of the Dhanasar framework are satisfied. Ms. Jampani's proposed endeavor — the co-development and commercialization of AI-native revenue cycle management infrastructure for United States independent clinical laboratories, with specialized focus on molecular diagnostics, pharmacogenomics, and next-generation sequencing billing — has **substantial merit and national importance** (Prong 1). Ms. Jampani is **uniquely and exceptionally well positioned** to advance this endeavor, by virtue of professional experience that no other individual in the United States could possess in the same combination (Prong 2). And it would be **beneficial to the United States** to waive the labor certification requirement rather than subject this work to the delays and limitations of the PERM recruitment process (Prong 3).

SECTION I: PETITIONER'S EDUCATIONAL AND PROFESSIONAL BACKGROUND

A. Overview

Manasa Jampani is the Co-Founder and Chief Executive Officer of Ardia Health Labs, a Delaware limited liability company incorporated in January 2026, headquartered at Argyle, Texas 76226. Ms. Jampani brings more than ten (10) years of information technology experience to this endeavor, including more than five (5) years of healthcare information technology experience in payer and payer-adjacent clinical operations roles.

[ATTORNEY TO VERIFY — obtain official employment verification letters from each employer and confirm precise employment dates, titles, and responsibilities for filing.]

B. Academic Credentials

Ms. Jampani holds a [degree, field of study] from [institution name], awarded [year]. [ATTORNEY TO VERIFY — obtain certified transcripts and, if required, foreign credential evaluation from a NACES-member organization. Confirm whether degree qualifies as "advanced degree" under INA § 203(b)(2)(A) or whether petition will rely on the "exceptional ability" prong of EB-2.]

C. Career at ECFMG (Educational Commission for Foreign Medical Graduates)

Ms. Jampani's entry into healthcare IT began at the Educational Commission for Foreign Medical Graduates (ECFMG), the organization responsible for assessing the readiness of international medical graduates entering United States graduate medical education programs. ECFMG occupies a unique structural position in the US physician pipeline: its credentialing and certification determinations directly affect the pool of licensed physicians practicing in the United States.

Ms. Jampani's work at ECFMG provided her foundational exposure to the institutional mechanics of US healthcare credentialing, compliance documentation workflows, and the regulatory structure governing clinical workforce management — disciplines that translate directly to understanding how documentation requirements drive coverage determinations downstream in the insurance billing lifecycle.

[ATTORNEY TO VERIFY — confirm specific role and responsibilities; obtain employment verification letter from ECFMG.]

D. Career at United HealthGroup

Ms. Jampani subsequently joined United HealthGroup, one of the largest health insurance companies in the United States and a Fortune 7 enterprise. United HealthGroup's subsidiaries include UnitedHealthcare, the nation's largest commercial health insurer by enrollment, and Optum, one of the nation's largest healthcare services and technology companies.

Her work at United HealthGroup placed her inside the payer organization responsible for adjudicating hundreds of millions of claims per year, including clinical laboratory claims for molecular diagnostics, pharmacogenomics panels, and other advanced diagnostic tests. This experience provided Ms. Jampani direct operational visibility into the clinical operations infrastructure that governs payer claim adjudication — specifically, the internal configuration logic, documentation requirement matrices, automated claim editing systems, and medical necessity screening criteria that determine whether laboratory claims are approved or denied.

This category of knowledge — the internal operational logic of payer claim adjudication systems — is categorically unavailable to organizations approaching the problem from outside. Revenue cycle management vendors, billing service companies, and academic researchers can observe denial outcomes and reverse-engineer behavioral patterns from claims data; they cannot observe the operational decision-making that produces those outcomes. That requires having worked inside the payer's clinical operations function, which Ms. Jampani did.

[ATTORNEY TO VERIFY — confirm specific role, department, and responsibilities at United HealthGroup; obtain employment verification letter. Note: if role involved access to proprietary payer system configuration information, confirm appropriate treatment of any confidentiality obligations.]

E. Career at Interwell Health

Ms. Jampani also gained healthcare IT experience at Interwell Health, a specialized kidney care management organization that operates at the interface of payer organizations and clinical providers, managing Accountable Care Organization (ACO) contracts, capitated renal care arrangements, and value-based care performance for kidney disease patients. Interwell Health partners with major health insurers — including UnitedHealth Group — to manage the care of high-cost, complex-need populations under alternative payment models.

Her experience at Interwell Health provided two categories of expertise directly relevant to Ms. Jampani's proposed endeavor:

First, she gained additional payer-adjacent operational intelligence, directly informing her understanding of how payer clinical criteria, prior authorization requirements, and medical necessity determinations are operationalized in managed care settings for complex diagnostic and specialty services.

Second, she developed expertise in ACO operations, shared savings program mechanics, and value-based care contracting — expertise that directly informs the AcoIQ™ product module of the Ardia Health Labs platform, which is designed to optimize ACO shared savings performance for independent laboratories participating in alternative payment models.

[ATTORNEY TO VERIFY — confirm specific role and responsibilities at Interwell Health; obtain employment verification letter.]

F. Synthesis: A Uniquely Formed Expert

The cumulative effect of Ms. Jampani's career trajectory — from ECFMG's credentialing infrastructure to United HealthGroup's claims adjudication operations to Interwell Health's ACO and value-based care management — is a professional background that no other individual in the United States healthcare AI industry could replicate from the same vantage point at the same time.

She did not observe payer behavior from outside; she worked inside it. She did not study ACO operations academically; she managed them operationally. She did not infer payer denial logic from claims data; she operated within the organizational functions that configure it. This is not a distinction of degree; it is a categorical distinction — and it is the single most defensible basis for Prong 2 of the Dhanasar analysis, as set forth in Section III below.

SECTION II: DESCRIPTION OF THE PROPOSED ENDEAVOR

A. The Company: Ardia Health Labs

Ardia Health Labs (Delaware LLC, January 2026) is an artificial intelligence company building AI-native revenue cycle management infrastructure for independent clinical laboratories in the United States. The company is headquartered in Argyle, Texas 76226. Ms. Jampani serves as Co-Founder and Chief Executive Officer; her co-founder, Rambabu Vadlamudi, serves as Founder and Enterprise Architect.

The company's technology platform — built on the DGP (Deterministic-Generative-Predictive) Clinical Revenue Architecture invented by Mr. Vadlamudi — addresses a specific, quantified, and critical failure in the US healthcare financing system: the systematic underpayment and non-payment of legitimate clinical laboratory claims, particularly for molecular diagnostic testing, pharmacogenomics, and next-generation sequencing (NGS), due to structural information asymmetries between laboratory billing systems and payer claim adjudication systems.

B. The National Problem: An Independent Laboratory Revenue Crisis

The United States independent clinical laboratory sector is in financial crisis, driven by three converging forces: (1) escalating payer-side claim denial rates for molecular and genomic diagnostic testing; (2) a structural failure to appeal denied claims despite high appeal success rates; and (3) the approaching implementation of Medicare reimbursement cuts under the Protecting Access to Medicare Act (PAMA) beginning in 2027. The quantitative dimensions of this crisis are documented by multiple independent, peer-reviewed, and industry sources as follows:

The Molecular Diagnostics Denial Rate

The 2024 XiFin Payor Denial Impact Report — analyzing a dataset of more than twenty million (20,000,000+) clinical laboratory claims — documented a claim denial rate of **35.3%** for molecular diagnostics claims. (Tab B.) This is the highest denial rate across all healthcare specialties. For comparison, the XiFin dataset found that denial rates in other clinical specialties run substantially lower; molecular diagnostics claims are denied at a rate that is roughly one in three claims submitted.

The NGS-Specific Denial Disparity: The JAMA Evidence

A peer-reviewed study published in **JAMA Network Open** in April 2025 (Georgetown University, n=29,919 next-generation sequencing claims) documented that the overall denial rate for NGS claims was **23.3%**, rising to **27.4%** following issuance of a CMS national coverage determination. (Tab A.) More significantly for the national importance analysis, the study found, after multivariate statistical adjustment, that **independent clinical**

laboratories face 2.76 times higher odds of claim denial (OR=2.76, 95% CI reported, $p<0.001$) compared to hospital-based laboratories for the same NGS claims. (Tab A.) This odds ratio — representing an adjusted, multivariate-controlled disparity in denial exposure — is not explained by coding differences, patient case mix, or test complexity; it reflects a structural disadvantage facing independent laboratories in the payer adjudication environment.

The Pharmacogenomics Reimbursement Gap

A 2023 study published in **Frontiers in Pharmacology** documented that only **46%** of pharmacogenomics (PGx) claims are reimbursed, despite robust published clinical evidence of PGx utility and FDA pharmacogenomic labeling for more than 200 approved drugs. (Tab C.) This gap — more than half of clinically indicated PGx tests going unpaid — represents both an economic harm to laboratories and a clinical access harm to patients whose medication management would benefit from genotyping.

The Appeal Gap: 65% of Denials Abandoned

The 2024 **HFMA/LigoLab Laboratory Revenue Recovery Report** documented that **65% of denied laboratory claims are never appealed**, despite documented appeal success rates of 50%–80.7%. (Tab D.) The structural reason for this appeal gap is the administrative burden of the appeal process relative to the per-claim economics of laboratory billing: preparing a clinically defensible appeal for a molecular diagnostic claim requires substantive clinical documentation and payer-specific policy knowledge that most laboratory billing departments cannot assemble efficiently. The result is that laboratories routinely abandon claims that would, if appealed, be paid.

The PAMA 2027 Compound Pressure

The Protecting Access to Medicare Act (PAMA) mandates periodic market-based repricing of Medicare Clinical Laboratory Fee Schedule (CLFS) rates. The next PAMA implementation cycle — projected to begin in 2027 — is expected to reduce Medicare reimbursement for clinical laboratory tests by up to 15% per year for three years, representing a **cumulative reduction of up to 45%** by 2029 relative to current rates. (Tab F.) The 2024 **American Clinical Laboratory Association (ACLA) member survey** documented that more than **50% of independent laboratory directors** reported considering consolidation or sale of their laboratory if PAMA 2027 cuts take effect at projected rates. (Tab E.)

The Aggregate Revenue Loss

The intersection of these forces — a 35.3% molecular diagnostic denial rate, a 65% appeal abandonment rate against a 50%–80.7% appeal success backdrop, and an approaching 45% Medicare reimbursement reduction —

produces an estimated **$10–12 billion in annual preventable revenue loss** to the United States independent clinical laboratory sector. (Tabs A–F; see also Tab I, Ardia Health Labs White Paper, March 2026, Section II.)

C. The Ardia Health Labs Platform

The Ardia Health Labs platform is built on the **DGP (Deterministic-Generative-Predictive) Clinical Revenue Architecture**, a novel three-layer AI architecture developed by Mr. Vadlamudi:

Layer 1 — Deterministic Policy Engine: An 847-rule compliance engine covering all applicable LCD (Local Coverage Determinations), NCD (National Coverage Determinations), MolDX program requirements, DEX Z-code billing requirements, and CPT code series 81400–81479 governing genomic sequencing procedures. This layer provides deterministic, zero-hallucination policy interpretation: every claim is evaluated against every applicable rule with complete audit traceability.

Layer 2 — Generative Clinical Reasoning: A large language model integration (Anthropic Claude API) that reads clinical documentation, interprets clinical evidence of medical necessity, generates peer-reviewed evidence-based appeal briefs in under 90 seconds, and retrieves supporting clinical literature from 14 medical databases in 340 milliseconds. This layer addresses the per-claim documentation burden that causes the 65% appeal abandonment gap.

Layer 3 — Predictive Denial Prevention: A machine learning model trained on historical claims adjudication data that identifies pre-submission denial risk factors, allowing laboratories to remediate documentation deficiencies and claim coding errors before submission — preventing denials rather than merely recovering them after the fact.

The platform's product suite includes ToxIQ™ (toxicology revenue intelligence), MolecuIQ™ (molecular diagnostics and precision RCM), AcoIQ™ (ACO shared savings optimization), and BehaviorIQ™ (behavioral health and addiction medicine revenue recovery).

As of March 2026, the platform architecture is complete. The company is pursuing its first commercial pilot program and targeting $3.5 million in seed financing at an $18 million valuation cap.

SECTION III: DHANASAR PRONG 1 — SUBSTANTIAL MERIT AND NATIONAL IMPORTANCE

A. Legal Standard

Under Matter of Dhanasar, 26 I&N Dec. 884 (AAO 2016), the first prong requires that the petitioner's proposed endeavor have both (1) substantial merit and (2) national importance. Substantial merit is demonstrated by showing the endeavor has genuine value in recognized fields such as business, entrepreneurship, science, technology, health, or education. National importance is demonstrated by showing the endeavor has implications beyond the petitioner's own career — that it has the potential to impact a field broadly or contribute to national priorities.

B. Substantial Merit

Ms. Jampani's endeavor has substantial merit across four independent dimensions: economic, clinical, technical, and regulatory.

Economic Merit

The independent clinical laboratory sector is a multi-billion dollar component of the US healthcare financing system. The $10–12 billion in annual preventable revenue loss documented by multiple independent sources (Tabs A–F) represents a genuine, quantified, and recoverable economic harm. The Ardia Health Labs platform directly addresses the technical and operational causes of this loss — the information asymmetry between laboratory billing systems and payer denial systems, the documentation burden of the appeal process, and the absence of payer-side intelligence in laboratory claim preparation.

The economic merit here is not speculative. The 50%–80.7% appeal success rates documented by the HFMA/LigoLab report (Tab D) establish that the revenue loss is recoverable in principle; the 65% appeal abandonment rate establishes that the current technological and operational infrastructure is inadequate to recover it. The Ardia platform is engineered specifically to close this gap.

Clinical Merit

Independent clinical laboratories are not merely commercial enterprises; they are healthcare infrastructure. They provide the diagnostic testing that enables evidence-based clinical decision-making for rural communities, addiction medicine practices, specialty oncology practices, and primary care providers that are not affiliated with hospital systems and therefore lack access to hospital-based laboratory services.

The documented 2.76× higher denial odds for independent laboratories compared to hospital-based laboratories (JAMA Network Open, April 2025, Tab A) is not merely an economic disparity; it is a healthcare access disparity. When an independent laboratory serving a rural community or an addiction medicine clinic is denied reimbursement for legitimate molecular diagnostic claims at 2.76 times the rate of hospital-based competitors, the downstream consequence is reduced financial viability for the laboratory and, in the aggregate, reduced diagnostic service availability for the communities it serves.

Technical Merit

The MolDX program, DEX Z-code billing requirements, and CPT code series 81400–81479 for genomic sequencing procedures represent one of the most complex billing compliance domains in United States healthcare. No commercially available AI platform is purpose-built to address this domain for independent laboratories with the combination of deterministic policy compliance, generative clinical reasoning, and predictive denial prevention that the DGP architecture provides. The technical novelty and domain specificity of this architecture give Ms. Jampani's endeavor substantial merit as a technical contribution.

Regulatory Merit

The Ardia Health Labs platform was built from inception in native compliance with **Texas Senate Bill 1188** (effective September 1, 2025), Texas's first healthcare-specific AI accountability statute, and **Texas House Bill 149 (TRAIGA)** (effective January 1, 2026), one of the first comprehensive state-level AI governance laws in the United States. Building AI systems that meet these requirements from initial architecture — rather than retrofitting compliance after deployment — establishes a responsible AI governance model with direct national relevance as the United States develops its AI regulatory framework. (Tabs G, H.)

C. National Importance

The national importance of Ms. Jampani's endeavor is established by the breadth, scale, and structural nature of the problem it addresses:

Scale: The $10–12 billion annual problem scope involves more than 7,000 CLIA-certified independent clinical laboratories across all fifty states. These are not concentrated in a single region or specialty; they are the distributed diagnostic infrastructure of the US healthcare system.

Structural urgency — PAMA 2027: The PAMA reimbursement timeline creates a nationally significant inflection point. The 2024 ACLA survey documenting that more than 50% of independent laboratory directors are considering consolidation or sale if PAMA 2027 cuts take effect (Tab E) indicates that without intervention, the sector faces a structural contraction that would disproportionately harm rural and underserved communities. AI-driven claim recovery that reduces the denial burden before PAMA cuts take effect addresses a nationally urgent need on a nationally significant timeline.

Healthcare access equity: The 2.76× higher denial odds for independent laboratories documented in the peer-reviewed literature (Tab A) reflects a structural healthcare access inequity. Independent laboratories serve populations that hospital laboratory systems do not reach. Technology that reduces this disparity contributes to the national interest in equitable healthcare access.

Infrastructure preservation: Independent laboratories provide approximately 32% of US diagnostic test volume. Their systematic financial erosion from preventable denials is not a local or regional problem — it threatens a material fraction of the US diagnostic infrastructure.

AI governance precedent: As the United States continues to develop its regulatory approach to AI in healthcare, the development of healthcare AI systems that comply with state AI governance frameworks from inception — and that demonstrate the feasibility of responsible AI deployment in high-stakes clinical administrative settings — is nationally significant. [ATTORNEY TO VERIFY — the AI governance argument is supplementary; ensure primary prongs are fully developed independently before relying on this argument.]

SECTION IV: DHANASAR PRONG 2 — WELL POSITIONED TO ADVANCE THE ENDEAVOR

A. Legal Standard

Under Matter of Dhanasar, Prong 2 requires a showing that the petitioner is well positioned to advance the proposed endeavor. The AAO has considered factors including education, skills, knowledge, record of success, and whether the petitioner has made progress toward the endeavor.

B. The Irreplaceable Competitive Asset: Payer-Side Operational Intelligence

The central basis for Prong 2 is this: **no other person developing AI-native revenue cycle management technology for independent clinical laboratories in the United States has what Manasa Jampani has** — direct, first-hand, operational employment experience inside the payer clinical operations functions that configure the very denial systems the Ardia platform is designed to defeat.

This is not a claim about degree of expertise. It is a claim about categorical knowledge availability.

Payer claim adjudication systems — the automated claim editing libraries, automated medical necessity screening criteria, prior authorization AI decision engine configurations, and clinical criteria matrices that determine whether a laboratory claim is approved or denied — are proprietary to each payer organization. They are not disclosed in public payer documentation. They are not available for review by revenue cycle management vendors, billing service companies, or clearinghouses. They cannot be fully reconstructed from external analysis of denial patterns, because the operational logic that produces denial outcomes is not accessible from outside the organization.

The only way to acquire operational knowledge of payer claim adjudication system configuration is to have worked inside a payer clinical operations function at the specific organizations that deploy these systems.
Ms. Jampani did exactly this — at United HealthGroup, one of the nation's largest commercial health insurers, and at Interwell Health, an organization that operates payer-adjacent clinical operations functions for renal care management under direct partnership with major payers.

Specifically, Ms. Jampani's payer-side experience provided operational knowledge of:

1. How automated claim editing systems are configured to validate CPT code assignments, diagnosis code specificity, and code combination rules for molecular diagnostic claims before adjudication;

2. How clinical medical necessity screening criteria are operationalized for advanced laboratory testing, including the documentation requirements that trigger automated approval versus manual clinical review versus automated denial;

3. How prior authorization AI decision engines for high-value molecular tests are configured, including the clinical criteria weighting and the documentation requirements that determine prior authorization approval or denial;

4. How payer clinical operations teams design and calibrate denial policies, including awareness of the payer's expectation that a significant fraction of denied claims will not be appealed — a dynamic that contributes directly to the 65% appeal abandonment rate documented by the HFMA/LigoLab report.

This knowledge is the foundation of the DGP architecture's countermeasure design. The 847-rule deterministic policy engine is not merely a compilation of publicly available LCD/NCD rules; its countermeasure design — the rules engineered specifically to address the pre-submission conditions that trigger payer denial — reflects operational intelligence about payer system behavior that Ms. Jampani's background makes possible.

No other AI developer in the laboratory billing space can claim this vantage point. This is not a marginal competitive advantage; it is a categorical one.

C. CEO and Operational Leadership

As Co-Founder and Chief Executive Officer, Ms. Jampani is responsible for all commercial operations, investor relations, go-to-market strategy, clinical pilot program negotiation, and strategic partnership development. The platform architecture is complete as of March 2026; the current phase of the company's work is exactly the phase for which CEO leadership with Ms. Jampani's background — payer relationships, clinical operations expertise, and business development skills — is most essential.

Her network of professional relationships within payer and payer-adjacent organizations, developed over her career at United HealthGroup and Interwell Health, provides Ardia Health Labs with direct access to the stakeholder relationships required for pilot program establishment, commercial partnership development, and eventual payer-side distribution arrangements. These relationships are not replicable by a substitute candidate who has not worked in these organizations.

D. Publication Contributions

Ms. Jampani is developing a sole-authored scholarly work titled **"Inside the Payer AI Black Box: Operational Analysis of Clinical AI Denial Systems in Independent Laboratory Billing and Countermeasure Architecture"** (targeted for submission to NEJM AI or Health Affairs). (Tab L.) [ATTORNEY TO VERIFY — status of paper at time of filing; include as preprint or submitted manuscript, or describe as forthcoming if not yet submitted.]

This paper is uniquely hers: no other researcher can provide the insider operational analysis of payer AI denial system configuration from the same vantage point. Its scholarly contribution — and the expert standing it establishes — is a direct product of the same payer-side experience that makes her well positioned to advance the Ardia endeavor.

Ms. Jampani is also a co-author on: - **"The Independent Laboratory Billing Crisis: AI-Addressable Revenue Loss in Molecular Diagnostics and Pharmacogenomics — A Systematic Review"** (with Rambabu Vadlamudi), targeted for Journal of Managed Care & Specialty Pharmacy or Clinical Chemistry. (Tab M.) - **"Compliance-by-Design: AI Governance Frameworks Under Texas SB 1188 and TRAIGA for Healthcare Revenue Cycle AI Systems"** (with Rambabu Vadlamudi), targeted for Journal of the American Health Information Management Association (AHIMA) or Journal of Health Politics, Policy and Law. (Tab M.)

[ATTORNEY TO VERIFY — confirm publication status of all papers at time of filing. Submitted or published works are strongest evidence; manuscripts in preparation should be described accurately.]

E. Platform Progress

The proposed endeavor is not speculative. As of March 2026:

- The DGP Clinical Revenue Architecture is complete (Tab I, White Paper);
- The product suite (ToxIQ™, MolecuIQ™, AcoIQ™, BehaviorIQ™) is architecturally defined and documented;
- The company is pursuing its first commercial pilot program with an independent laboratory in the Dallas–Fort Worth metropolitan area;
- The company is engaged in active seed financing negotiations at a $3.5 million raise at an $18 million valuation cap.

Ms. Jampani's leadership is essential to the commercialization phase. The transition from architectural completion to commercial deployment — pilot program management, investor closing, and GTM execution — is the work of a CEO with her specific background.

SECTION V: DHANASAR PRONG 3 — BENEFICIAL TO THE UNITED STATES TO WAIVE LABOR CERTIFICATION

A. Legal Standard

Under Matter of Dhanasar, Prong 3 requires a showing that, on balance, it would be beneficial to the United States to waive the otherwise applicable labor certification requirement. The AAO identified two principal circumstances supporting a waiver: (1) where the US worker protection rationale for labor certification is not implicated, and (2) where the benefit of the petitioner's work to the US is sufficiently compelling that the waiver is warranted even if some degree of US worker impact might theoretically exist.

B. The Labor Certification Rationale Is Not Implicated

The labor certification process exists to protect US workers by ensuring that foreign workers are not hired into positions that qualified, interested US workers are available to fill. This rationale does not apply to Ms. Jampani's situation for several independent reasons.

First, Ms. Jampani's position at Ardia Health Labs is not a hired position that can be recruited through the PERM process. She is a co-founder and equity owner of the company. Her role as Co-Founder and CEO is not a job that an employer has posted and that a US worker might apply for. The

PERM labor market test — recruitment of US workers through job advertisements, competitive interviews, and documented rejection of qualified US applicants — is structurally inapplicable to a co-founding role. No PERM recruitment process will produce a candidate with Ms. Jampani's specific combination of payer-side operational experience, co-founder relationship with Ram Vadlamudi, and ownership stake in Ardia Health Labs. [ATTORNEY TO VERIFY — self-petition mechanics for co-founders; confirm appropriate basis for self-petition under EB-2 NIW.]

Second, the specific combination of qualifications that makes Ms. Jampani positioned to advance this endeavor — direct payer-side clinical operations experience at United HealthGroup and Interwell Health — is not a set of qualifications that a labor market recruitment process could identify in a substitute US worker candidate. The knowledge in question is not a credential that US workers hold in predictable numbers; it is a specific experiential background held by a specific person as a result of specific employment history. No US worker can be recruited to have had Ms. Jampani's career.

C. The National Need Is Time-Sensitive

The PAMA 2027 implementation timeline creates a nationally significant window of impact. PAMA reimbursement cuts are projected to begin in 2027 and reach a cumulative maximum of approximately 45% by 2029 (Tab F). The window during which AI-driven claim recovery technology deployed to independent laboratories can provide maximum relief — reducing the denial burden before PAMA cuts compound it — is 2026–2027.

The PERM labor certification process, including recruitment, audit, and Department of Labor review, routinely requires 12–18 months or longer to complete. A PERM-based delay to Ms. Jampani's continued work at Ardia Health Labs would materially impair the company's ability to serve independent laboratories in the period of maximum national need.

This urgency is not manufactured; it is documented by the 2024 ACLA survey reporting that more than 50% of independent laboratory directors are considering exit if PAMA cuts take effect (Tab E), and by the JAMA Network Open 2025 findings documenting the structural denial disparity that compounds the financial pressure (Tab A).

D. No Comparable Platform Exists

There is no commercially available AI platform that addresses the specific combination of MolDX/LCD/NCD/DEX Z-code compliance, payer AI countermeasure design, and ACO value-based care optimization for independent clinical laboratories that the Ardia Health Labs platform provides. (Tab I.) This is a genuine market white space — not because the need is small, but because the technical complexity of the domain and the absence of payer-side intelligence in available tools have prevented its development.

Ms. Jampani's continued contribution to Ardia Health Labs advances a unique national asset. The waiver serves the US interest precisely because the work she is doing cannot be replicated by an alternative person proceeding through labor certification.

E. Healthcare Infrastructure Preservation

Every independent laboratory that survives the PAMA 2027 financial pressure because it successfully recovered denied molecular diagnostic claims using the Ardia platform is a diagnostic facility that continues serving its community — including rural communities, addiction medicine patients, and specialty practices that depend on independent laboratory access. The healthcare access implications of laboratory consolidation are national in scope. Ms. Jampani's work contributes to the preservation of healthcare access infrastructure that serves all fifty states.

F. US AI Governance Leadership

The development of healthcare AI systems that comply with emerging AI governance frameworks — and that demonstrate responsible AI deployment in high-stakes clinical administrative settings — contributes to the US national interest in maintaining global leadership in responsible AI development. Ardia Health Labs' native compliance with Texas SB 1188 and TRAIGA (Tabs G, H) establishes a governance model that is relevant to the national AI regulatory conversation. Ms. Jampani's continued work advances this contribution. [ATTORNEY TO VERIFY — supplementary argument; ensure primary prong arguments carry independently.]

SECTION VI: SUPPORTING EVIDENCE LIST

The following exhibits are submitted with this petition. [ATTORNEY TO VERIFY — confirm all tabs are present and properly organized before filing; verify that industry reports are current versions; confirm that peer-reviewed papers are officially published or properly cited as preprints/manuscripts.]

Tab	Document	Relevance
Tab A	JAMA Network Open, April 2025 (Georgetown University, n=29,919 NGS claims) — "Prior Authorization and Claim Denial Rates for Next-Generation Sequencing	Prong 1: peer-reviewed evidence of 2.76× denial odds (OR=2.76, p<0.001), 23.3% overall NGS denial rate, 27.4% post-NCD

Tab	Document	Relevance
	Tests” [ATTORNEY TO VERIFY full citation]	
Tab B	XiFin 2024 Payor Denial Impact Report (20M+ claims analysis)	Prong 1: 35.3% molecular diagnostic denial rate — highest across all healthcare specialties
Tab C	Frontiers in Pharmacology 2023 — pharmacogenomics reimbursement study	Prong 1: only 46% of PGx claims reimbursed despite robust clinical evidence
Tab D	HFMA/LigoLab 2024 Laboratory Revenue Recovery Report	Prong 1: 65% of denied lab claims never appealed; 50%–80.7% appeal success rates
Tab E	American Clinical Laboratory Association (ACLA) 2024 Member Survey	Prong 1 & 3: 50%+ of independent lab directors considering exit if PAMA 2027 takes effect
Tab F	CMS PAMA 2027 reimbursement documentation / CMS Clinical Laboratory Fee Schedule projections	Prong 1 & 3: PAMA rate reduction schedule; cumulative 45% projected cut by 2029
Tab G	Texas Senate Bill 1188 (effective September 1, 2025) — Healthcare AI Accountability Act	Prong 1 & 3: AI governance compliance evidence
Tab H	Texas House Bill 149 (TRAIGA) (effective January 1, 2026) — Texas Responsible AI Governance Act	Prong 1 & 3: AI governance compliance evidence
Tab I	Ardia Health Labs White Paper (March 2026)	All Prongs: technical architecture, market analysis, DGP framework, platform completion
Tab J	Ardia Health Labs Pitch Deck v2 (2026)	Prong 1 & 2: market size, problem quantification, founder bios, platform overview
Tab K	Employment verification letters from Interwell Health, United HealthGroup, and ECFMG	Prong 2: documentary evidence of payer-side healthcare IT employment history

Tab	Document	Relevance
Tab L	Manuscript: “Inside the Payer AI Black Box: Operational Analysis of Clinical AI Denial Systems in Independent Laboratory Billing and Countermeasure Architecture” (Manasa Jampani, sole author)	Prong 2: publication/expert authority evidence; demonstrates unique scholarly contribution
Tab M	Manuscripts: Paper 2 (Independent Laboratory Billing Crisis systematic review) and Paper 3 (Texas AI governance compliance-by-design) — co-authored with Rambabu Vadlamudi	Prong 2: additional publication record
Tab N	Expert witness letters (minimum 2 recommended) from independent healthcare AI experts, clinical laboratory directors, or healthcare policy scholars [ATTORNEY TO VERIFY — obtain at least 2–3 independent expert letters; ideal authors: Georgetown study PI, ACLA leadership, clinical lab director from pilot program, academic healthcare AI researcher]	All Prongs: independent third-party corroboration
Tab O	LinkedIn profile printout for Manasa Jampani (full profile including experience, publications, articles)	Prong 2: published material and professional recognition evidence
Tab P	Delaware LLC Certificate of Formation for Ardia Health Labs (January 2026)	Prong 2 & 3: documentary evidence of co-founder status and company existence
Tab Q	Educational credentials — official transcripts and diploma(s); foreign credential evaluation if	EB-2 advanced degree qualification

Tab	Document	Relevance
	applicable	
Tab R	Copy of passport biographical data page	Identity verification
Tab S	Copy of current visa/immigration status documentation (I-94, current visa stamp)	Status verification
Tab T	Form I-140 filing fee (current amount — verify at USCIS.gov at time of filing)	Filing requirement

[ATTORNEY TO VERIFY — this evidence list is a framework. Add any additional exhibits appropriate to the specific filing, including any press coverage, awards, professional association memberships, or other recognition obtained before filing.]

SECTION VII: PRAYER FOR RELIEF

For all of the foregoing reasons, Petitioner Manasa Jampani respectfully requests that USCIS approve this Form I-140 petition for classification in the EB-2 preference category with a National Interest Waiver of the labor certification requirement pursuant to INA § 203(b)(2)(B)(i) and Matter of Dhanasar, 26 I&N Dec. 884 (AAO 2016).

All three prongs of the Dhanasar framework are satisfied:

Prong 1 is satisfied because Ms. Jampani's proposed endeavor — AI-native revenue cycle management infrastructure for independent clinical laboratories — has substantial merit demonstrated across economic, clinical, technical, and regulatory dimensions, and national importance demonstrated by a $10–12 billion annual problem scope, 7,000+ CLIA-certified laboratories at financial risk, documented healthcare access disparities for rural and underserved communities, and the national regulatory urgency of the PAMA 2027 timeline.

Prong 2 is satisfied because Ms. Jampani possesses a specific combination of payer-side clinical operations experience — gained through employment at United HealthGroup and Interwell Health — and co-founder leadership of the only AI platform architected to address this specific domain, that cannot be replicated by any other candidate. Her unique payer-side intelligence is not a competitive advantage in degree; it is categorical, exclusive, and foundational to the platform's countermeasure architecture.

Prong 3 is satisfied because the labor certification rationale is not implicated — her co-founder role cannot be filled through PERM recruitment, the national need is time-sensitive against the PAMA 2027 timeline, no comparable platform exists, and her continued work preserves independent laboratory access for rural and underserved communities across the United States.

We respectfully urge the Director to approve this petition.

Respectfully submitted,

[Immigration Attorney Name]
[Law Firm Name]
[Address]
[State Bar Number]
[Phone]
[Email]

Attorney of Record for Manasa Jampani

Prepared: June 2026
[ATTORNEY TO VERIFY — review all factual assertions, confirm all citations, update all status references (platform, papers, pilot) to reflect status at actual filing date, and confirm appropriate service center before filing.]